



February 23, 2026

Department of Health and Human Services
Office of the Assistant Secretary for Technology Policy /
Office of the National Coordinator for Health Information Technology
Washington, D.C.

Re: Request for Information– Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

Cedar Cares, Inc. (“Cedar”) appreciates the opportunity to respond to HHS’s Request for Information on Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care. Cedar strongly supports the Administration’s goal of positioning the United States as a global leader in artificial intelligence innovation while removing unnecessary regulatory barriers that impede responsible deployment of AI across healthcare.

Cedar is a healthcare technology company dedicated to improving the healthcare financial experience. Founded in 2016, Cedar’s platform unifies patient billing, payments, insurance coverage, and support into a single solution —driving improved results for healthcare providers and better financial journeys for patients. Cedar helps patients access the coverage, aid, and payment options they need with confidence. To date, Cedar has supported more than 50 million patients nationwide. Cedar’s platform uses AI and machine learning to help solve the growing complexity of patient financial engagement and deliver personalization at scale. Our experience deploying AI-enabled tools in regulated healthcare environments informs the comments below.

I. Regulatory and Programmatic Changes to Incentivize AI Adoption

Cedar encourages HHS to prioritize regulatory clarity, interoperability reforms, and targeted modernization efforts that enable AI tools to access high-quality data in a predictable and scalable manner. In many cases, barriers to AI adoption stem from



fragmented data access, artificial interoperability barriers created by incumbents, duplicative administrative requirements, and uncertainty regarding permissible automated access under existing rules.

HHS can accelerate responsible AI deployment by:

- Reinforcing that automated and AI-enabled access to health information via standardized APIs is permissible when consistent with HIPAA and information blocking safeguards, and ensuring access via those APIs is available and does not include discriminatory pricing (i.e., without disguised information blocking);
- Enforcing the adoption of HL7 FHIR standards across the healthcare ecosystem to cover a broader range of healthcare data, ensuring consistent and secure data exchange. While USCDI v3 was recently incorporated into the standards (which we applaud), actual implementation of the FHIR standards still remains fragmented and incomplete, especially for some types of data, like healthcare billing;
- Continuing to modernize and streamline certification and compliance requirements that no longer reflect current technical realities; and
- Aligning AI policy with broader interoperability initiatives, including CMS's patient-directed access and data exchange efforts, and HHS's initiatives to stop healthcare data information blocking.

Providing clear, operational guidance will reduce ambiguity for providers and technology developers while maintaining appropriate privacy and security protections.

II. Governance and Accountability Considerations for Non-Medical Device AI

For AI tools that are not regulated as medical devices, governance challenges typically relate to transparency, accountability, and contractual risk allocation rather than direct clinical safety oversight. In Cedar's experience, these considerations are



often effectively addressed through contractual safeguards, audit, and defined use-case limitations.

HHS can support innovation by clarifying baseline expectations around transparency, documentation, auditability, and data stewardship, while avoiding rigid or default liability frameworks that could discourage responsible AI development. Clear, risk-based guidance would allow AI-enabled tools to evolve while preserving accountability.

III. Supporting Private-Sector Certification and Testing

Cedar appreciates HHS's interest in supporting private-sector accreditation, certification, and testing frameworks. Industry-led benchmarking and validation initiatives are often better positioned to evolve alongside rapidly advancing AI technologies and adapt to emerging clinical and operational use cases.

HHS can play a constructive role by encouraging voluntary, consensus-driven standards and facilitating collaboration across stakeholders, rather than imposing static or prescriptive certification regimes that may quickly become outdated.

IV. Interoperability as a Catalyst for AI Innovation

Enhanced interoperability remains one of the most effective levers available to accelerate AI adoption in clinical care. Standardized, API-based access to longitudinal clinical and coverage data enables AI tools to support care coordination, operational efficiency, and patient engagement.

Cedar encourages HHS to ensure that digital health companies and application developers that directly support patients in all aspects of their medical care (including tools that enable them to afford and pay for their care) are able to participate meaningfully in health information exchange networks under clear, objective, and consistently applied criteria. Interoperability frameworks should promote patient-directed access to health information through applications of a patient's choice and prevent contractual, pricing, or technical practices that functionally limit or block participation.



Policies that reduce gatekeeping and promote fair, reasonable, and non-discriminatory access to interoperable data will expand market opportunities, fuel research and development, and advance patient-centered innovation.

Conclusion

Cedar supports HHS's efforts to accelerate the responsible adoption of AI through interoperability, regulatory clarity, and modernized governance frameworks. Thoughtful policy refinement can enable innovation while maintaining strong privacy, security, and accountability safeguards.

Cedar appreciates the opportunity to provide these comments and welcomes continued engagement with HHS as these policy efforts advance.

Respectfully submitted,

A handwritten signature in black ink, appearing to read "Vanessa Gage", with a stylized flourish at the end.

Vanessa Gage
General Counsel & Chief Compliance Officer
Cedar Cares, Inc.